Patient Encounter Performance Rubric

How to use

Rate the student's performance as supported by the encounter. Rate only what the scenario makes possible to observe; use N/A when a criterion is not appropriate.

Score	Label	Meaning
4	Strong	Consistent, effective, appropriate
3	Competent	Usually effective; minor gaps
2	Developing	Partly effective; important gaps
1	Unsafe or absent	Not demonstrated, misleading, or harmful
N/A	Not applicable	Domain could not be observed in this scenario

Domains

Score each domain from 4 to 1, or N/A, in the Score column.

Domain	4 Strong	3 Competent	2 Developing	1 Unsafe or absent	Score
1. Opening and purpose	Introduces self and role; establishes purpose and agenda; confirms the patient's immediate concern; sets expectations.	Opens clearly and establishes a workable focus.	Opening is understandable but misses role, agenda, or patient priorities.	Begins abruptly, creates confusion, or lacks a clinically appropriate purpose.	
2. Rapport, respect, and empathy	Uses attentive, nonjudgmental, culturally responsive language; acknowledges	Consistently respectful and responsive; patient appears heard.	Generally respectful but responses feel formulaic, rushed, or	Dismissive, judgmental, confusing, or emotionally unsafe	

Domain	4 Strong	3 Competent	2 Developing	1 Unsafe or absent	Score
	emotion and patient perspective; builds trust.		insufficiently attuned.	communicatio n.	
3. Information gathering	Uses open questions, focused follow-up, clarification, and appropriate sequencing; explores relevant history, context, risks, and goals.	Gathers most relevant information with logical follow-up.	Gathers some relevant information but is incomplete, disorganized, or overly leading.	Misses essential information or cannot sustain a useful history.	
4. Listening and response to cues	Recognizes verbal cues, uncertainty, concerns, questions, and changes in the patient's story; avoids interrupting or ignoring information.	Tracks the patient's narrative and follows up appropriately.	Responds to obvious cues but overlooks subtler concerns or inconsistencies .	Ignores, contradicts, or prematurely redirects important information.	
5. Explanation and shared decisions	Explains findings or next steps clearly; checks understanding; invites questions; incorporates preferences and readiness when relevant.	Provides clear, patient-center ed explanations and checks understanding.	Explanation is partly clear but uses jargon, gives limited rationale, or does not check understanding.	Provides inaccurate, overly technical, coercive, or absent explanation when needed.	

Domain	4 Strong	3 Competent	2 Developing	1 Unsafe or absent	Score
6. Clinical reasoning and synthesis	Identifies important findings; distinguishes relevant from irrelevant information; recognizes red flags or uncertainty; forms a plausible assessment or differential when appropriate.	Reasoning is clinically coherent and appropriately cautious.	Recognizes some key findings but misses connections, alternatives, or uncertainty.	Reasoning is unsupported, unsafe, internally inconsistent, or absent when required.	
7. Professionalism, safety, and closure	Maintains boundaries; protects privacy; communicates limitations; summarizes; confirms next steps, follow-up, or escalation when relevant.	Professional and safe throughout; closes with a clear shared plan.	Mostly professional but closure, limitations, or safety-netting is incomplete.	Creates safety risk, overstates certainty, breaches boundaries, or ends without an appropriate plan.	

Scoring

Total: ____ / 28 · N/A domains: ____

Use the total as a discussion aid, not a pass/fail grade. A score of 1 in professionalism, safety, or clinical reasoning (domains 6 and 7) should trigger review even when the total is high.

Evidence from the encounter

Note	Evidence
Strength demonstrated	

Note	Evidence
Important gap or missed opportunity	
Safety or accuracy concern	